DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 25–36.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 25: The front desk

Setting: Malia, 41, a hotel front desk clerk and mother of two, at a community mental health clinic on her doctor's referral. Her partner, Kekoa, joins for part of the visit.

> *"My doctor said I should talk to someone. I'm not crazy. I just haven't slept since they called about the trial."*

If asked about…

When did this start?	The robbery was nine months ago. I was shaky for about a week, then fine for months. About seven weeks ago the detective called with the trial date, and it all came back, worse than before.
What happened?	A man came in around three in the morning, pointed a gun at me, and said if I moved he'd shoot. He took the drawer and ran. Maybe a minute. They caught him two weeks later.
Memories and dreams	I see his face when I close my eyes. Most nights I dream about the gun and wake up sweating. If someone in a hoodie walks into the lobby, my heart pounds.
What you avoid	I asked for day shifts. I drive the long way so I don't see the hotel. I don't want to talk about it, even with Kekoa.
How you see yourself	I should have seen him coming. I should have locked the side door. It was my fault. I feel cut off, like there's a wall between me and my kids.
Sleep	Maybe four hours, broken. I check the locks twice before bed. I sit facing the door wherever I go.
Alcohol, drugs, medications	I don't drink, never have. No drugs. Just vitamins.
Medical history	Healthy. My doctor did a checkup and bloodwork last month, thyroid too. All normal.
Thoughts of suicide or harming others	No. I'm scared, I'm not wanting to die. My kids need me. I've never hurt myself, and I don't want to hurt anyone, not even him.
Work and family	I've called out six times in the last month. I yell at my kids over shoes on the floor and then I cry about it. I miss being the fun mom.
Family history	My dad came back from the Army quiet and jumpy. Nobody ever called it anything.
Culture and context	My aunty wants our family to do ho'oponopono, to talk it through and set things right. I'm open to it. I also just need to sleep.

Only if asked (don’t volunteer)

- Was well for months after the robbery; symptoms began about seven weeks ago when the trial date was set
- Blames herself for the robbery
- Moved to day shifts and drives the long way to avoid the hotel
- Normal checkup and thyroid last month; no alcohol or drugs

Patient 25: answer and debrief

DIAGNOSIS

Posttraumatic stress disorder

An armed robbery with a death threat is a qualifying trauma. Clue 3 flips the lead toward an adjustment reaction to the trial, because she seemed well for months. Clues 4 and 5 flip it back: intrusive memories and dreams, avoidance, self-blame, detachment, hypervigilance and startle, all tied to the robbery and lasting more than 1 month. Full criteria were not met until more than 6 months after the event, so the specifier is 'with delayed expression'. Exam lesson: a quiet stretch after a trauma does not rule out PTSD, and acute stress disorder applies only from 3 days to 1 month after the event.

What had to be ruled out

- Acute stress disorder: the robbery was nine months ago; that diagnosis covers only 3 days to 1 month after a trauma
- Adjustment disorder: full PTSD criteria are met, and adjustment disorder is a residual category
- Major depressive disorder: fear and arousal rather than pervasive low mood; no wish to die
- Substance/medication or medical cause: no alcohol or drugs; normal checkup and thyroid last month

Checkpoint: What would you ask or check NEXT?

A. Reassure her that stress before a trial is normal and should fade once the case is over

B. Ask about symptoms tied to the robbery itself: memories, dreams, reminders she avoids, and how she sees herself since then **(best)**

C. Help her ask the prosecutor to excuse her from testifying

Symptoms that begin months after a trauma can still be PTSD, and a new reminder such as a trial often sets them off, so assess the trauma symptom clusters before deciding. A assumes a passing stress reaction without asking. C is advocacy she may want later, but it acts on an untested assumption and skips the assessment.

The case in six clues (as the app reveals it)

1. I can't sleep, I snap at my kids over nothing, and I've called out of work six times in the last month.
2. Eight months ago a man pointed a gun at me across the front desk and took the cash drawer. He said if I moved, he'd shoot me.
3. Kekoa: Right after, she was shaky for about a week, then went back on nights and was her old self for months. This only started about seven weeks ago, when the detective called to say the trial date was set.
4. Now I see his face when I close my eyes, and most nights I wake up from dreams about the gun. I asked my manager to move me to days, and I drive the long way so I don't pass the hotel.
5. I keep thinking I should have seen him coming, that it was my fault. I sit facing the door everywhere, I jump at every slammed car door, and I feel cut off from Kekoa and the kids, like I'm behind a wall.
6. I'm not sad all the time and I don't want to die; I'm scared. I don't drink or use anything, and my checkup last month, thyroid included, was normal.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Generalized anxiety disorder
2. Posttraumatic stress disorder, Adjustment disorder, Major depressive disorder
3. Adjustment disorder, Posttraumatic stress disorder, Major depressive disorder
4. Posttraumatic stress disorder, Adjustment disorder
5. Posttraumatic stress disorder, Adjustment disorder, Major depressive disorder
6. Posttraumatic stress disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder

Patient 26: The superintendent

Setting: Wendell, 58, the live-in superintendent of an apartment building, at an emergency shelter set up in a school gym. A disaster mental health volunteer meets him; his wife, Joyce, sits with him.

> *"I'm fine. Go check on the families from the third floor; they lost everything. I just need some sleep."*

If asked about…

When did this start?	Twelve days ago, the night of the fire. It started right then and hasn't let up.
What happened?	The alarms went off around two in the morning. I went in three times and got people out. I couldn't get to Mrs. Alvarez. The firefighters brought her out, but she didn't make it.
Memories and dreams	Anything burning, even toast, and I'm back in the stairwell. I dream about it and wake up gasping.
Feeling unreal; memory	Everything's behind dirty glass. There's maybe an hour that night I can't find. I remember the third trip in, then I'm sitting on the curb with a blanket.
What you avoid	I won't walk down our block. When Joyce brings it up, I walk out of the room.
Sleep and concentration	Two or three hours a night on a cot. I read the same aid form four times and still can't fill it out.
Alcohol, drugs, medications	Sober twenty years. No drugs. Just my blood pressure pill, same dose for years, and I've kept taking it.
Medical history	The ER checked my lungs and did the carbon monoxide test. All fine. I didn't hit my head. My blood pressure is controlled.
Thoughts of suicide or harming others	No, I'm not going to hurt myself or anybody. I keep thinking I should have gotten to her. But Joyce and my grandson need me.
Mood and enjoyment	I can't feel anything good. The ballgame was on and I just stared at it. I'm short with everybody.
Family history	Nobody in my family had mental health problems that I know of. My mother called it nerves when anyone was upset.
Culture and context	I've been the super there twenty-two years. Those tenants are my people. My pastor came by the shelter and prayed with us. Asking for help isn't something I do.

Only if asked (don’t volunteer)

- The fire was twelve days ago
- About an hour of that night he cannot remember
- Avoids his block and any talk of the fire
- Feels he should have reached Mrs. Alvarez

Patient 26: answer and debrief

DIAGNOSIS

Acute stress disorder

Twelve days after the fire, he has more than the 9 of 14 symptoms needed across intrusion, negative mood, dissociation, avoidance and arousal. Clue 3 flips the lead from PTSD: it has been less than 1 month, inside the 3 days to 1 month window of acute stress disorder. A normal carbon monoxide level and no head injury make a medical cause of the memory gap unlikely. If symptoms last past 1 month and meet criteria, the diagnosis becomes PTSD. Exam lesson: early care is Psychological First Aid, not forced debriefing; offer trauma-focused CBT if symptoms persist.

What had to be ruled out

- Posttraumatic stress disorder: less than 1 month since the fire
- Adjustment disorder: the stressor was traumatic and full acute stress disorder criteria are met
- Depersonalization/derealization disorder: the unreal feeling began with the fire and is part of the acute stress picture
- Medical cause (smoke inhalation, carbon monoxide, head injury): normal ER checks, no head injury
- Substance cause: sober twenty years

Checkpoint: What would you ask or check NEXT?

A. Have him retell the night of the fire in full detail now so he can process it

B. Check his immediate needs and safety: injuries, smoke exposure, sleep, alcohol use, and any thoughts of harming himself, and offer practical help **(best)**

C. Explain that he has PTSD and refer him to a long-term trauma clinic

In the first weeks after a disaster, Psychological First Aid comes first: safety, medical needs, calm, practical help and a check for risk. A is single-session debriefing, which has not been shown to prevent PTSD and may make some people worse. C is wrong on timing: PTSD needs symptoms for more than 1 month.

The case in six clues (as the app reveals it)

1. Everything feels unreal, like I'm watching my life through dirty glass. I can't sleep, and I can't think straight.
2. Joyce: Since the fire in our building he hasn't been himself. He went back in three times for tenants, and Mrs. Alvarez on the fourth floor didn't make it out.
3. That was twelve days ago. Anything burning, even toast, puts me right back in that stairwell, and I wake up choking from dreams about it.
4. There's maybe an hour of that night I can't remember at all. I won't walk down our block, and when Joyce brings up the fire, I leave the room.
5. The ER checked my lungs and my carbon monoxide level that night, and both were fine. I didn't hit my head, and I've been sober twenty years.
6. Joyce: He jumps at every siren, he snapped at our grandson, and he can't enjoy anything, not even the ballgame. That's not my husband.

What a skilled clinician weighs after each clue

1. Depersonalization/derealization disorder, Posttraumatic stress disorder, Acute stress disorder, Major depressive disorder
2. Posttraumatic stress disorder, Acute stress disorder, Depersonalization/derealization disorder
3. Acute stress disorder, Adjustment disorder, Depersonalization/derealization disorder
4. Acute stress disorder, Adjustment disorder
5. Acute stress disorder, Adjustment disorder
6. Acute stress disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Acute Stress Disorder; National Child Traumatic Stress Network and National Center for PTSD (2006), Psychological First Aid: Field Operations Guide, 2nd ed.; Rose, Bisson, Churchill & Wessely (2002), Cochrane Database of Systematic Reviews, Psychological debriefing for preventing post traumatic stress disorder

Patient 27: The bakery shift

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tevita, 44, a shift supervisor at a commercial bakery and father of three, at his first employee assistance program (EAP) visit.

> *"HR gave me this number. I've never done anything like this. I've been a mess since my wife left, and now it's showing at work."*

If asked about…

When did this start?	Seven weeks ago, the day she moved out with our youngest. I was fine before that. Tired from work, but fine.
Mood	Down most days. It hits like a wave when I walk into the quiet house, or when I see her side of the closet. Then it eases off.
Sleep	A few nights a week I wake at three and lie there. The other nights I sleep okay.
Appetite and energy	I eat fine; my mom and aunties keep bringing food. Energy's okay most days.
Interest	I still love coaching my son's soccer team on Saturdays. That's the best part of my week. I still watch the games with my brothers.
Self-worth and guilt	I don't feel worthless. I wonder what I missed in the marriage, but I know I'm a good dad.
Thoughts of suicide	No. Never. My kids need me, and I'm not that low.
Alcohol, drugs, medications	A beer at family barbecues. No drugs. No medications.
Medical history	Healthy. My doctor did bloodwork last month, thyroid and blood count, all normal.
Work	I mixed up two orders and we lost a pallet of rolls. My manager wrote me up. I've called out twice. Before this I hadn't missed a day in nine years.
Family history	My dad had a hard time when his brother died, but nobody called it depression. No one in my family has seen a counselor.
Culture and context	In my family the men don't cry in front of people. My minister says to give her time and keep praying. I don't want my kids to see me like this.

Only if asked (don’t volunteer)

- His wife moved out seven weeks ago
- Appetite, enjoyment and self-worth are intact; no suicidal thoughts
- Written up at work and has called out twice
- Normal thyroid and blood count last month

Patient 27: answer and debrief

DIAGNOSIS

Adjustment disorder

A clear stressor seven weeks ago plus low mood points to adjustment disorder or major depression. Clue 3 pushes toward depression, but clue 5 flips it: appetite, enjoyment and self-worth are intact and there are no suicidal thoughts, leaving about 3 depressive symptoms, short of the 5 MDD needs. Onset within 3 months of the separation, plus a write-up and missed work, make this adjustment disorder with depressed mood rather than a normal reaction. Exam lesson: adjustment disorder is the residual call; if full MDD criteria are met after a stressor, diagnose MDD.

What had to be ruled out

- Major depressive disorder: about 3 symptoms; interest, appetite and self-worth intact; no suicidal thoughts
- Persistent depressive disorder: weeks, not 2 years
- No mental disorder (normal or expected reaction): the write-up and missed work show real impairment
- Depressive disorder due to another medical condition or substance cause: normal thyroid and blood count; minimal alcohol; no medications

Checkpoint: What would you ask or check NEXT?

A. Ask about the rest of the depression picture: interest, appetite, energy, self-worth and thoughts of suicide **(best)**

B. Suggest he ask his doctor about starting an antidepressant

C. Refer him and his wife to couples counseling

The full symptom count decides between major depression and adjustment disorder, and suicide risk is asked in every depression assessment; a PHQ-9 can help. B treats a diagnosis that has not been made. C may help later, but his wife has moved out and he came for himself, so it skips the assessment.

The case in six clues (as the app reveals it)

1. I'm down most days. I cry in my truck before my shift, and I can't keep my mind on the production schedule.
2. My wife moved out with our youngest seven weeks ago. She says she needs space. I didn't see it coming.
3. A few nights a week I wake at three and can't get back to sleep. Last week I mixed up two orders, and we lost a whole pallet of rolls.
4. My doctor checked my thyroid and blood count last month, and both were normal. I have a beer at family barbecues, that's it, and I'm not on any medications.
5. I still eat fine, and I still love coaching my son's soccer team on Saturdays. I don't feel worthless, and I've never thought about killing myself. It hits me like a wave when I walk into the quiet house.
6. My manager wrote me up for the order mix-up, and I've called out twice. Before she left, I hadn't missed a day in nine years.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Persistent depressive disorder
2. Adjustment disorder, Major depressive disorder, No mental disorder (normal or expected reaction)
3. Major depressive disorder, Adjustment disorder
4. Major depressive disorder, Adjustment disorder
5. Adjustment disorder, No mental disorder (normal or expected reaction)
6. Adjustment disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Adjustment Disorders; Kroenke, Spitzer & Williams (2001), Journal of General Internal Medicine 16(9), The PHQ-9: validity of a brief depression severity measure

Patient 28: The slippers by the door

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Thanh, 66, a retired postal carrier, at a behavioral health visit in her primary care clinic. Her daughter, Vy, made the appointment and comes with her.

> *"My daughter worries too much. I miss my husband. Is that a sickness now?"*

If asked about…

When did he die?	Four months ago, at home with hospice. Liver cancer. We had almost a year to prepare, but you are never ready.
What a day is like	Every morning I light incense and talk to his photo, tell him about the grandkids. Then I make breakfast for the little ones.
Crying	When I cook his favorite soup, or hear his songs. It comes like a wave, then it goes. Some days are better than others.
Sleep and appetite	Some nights I wake up and reach for his side of the bed. I eat a little less than before. The doctor weighed me today and said three pounds is nothing to worry about.
Self-worth and guilt	I was a good wife. I cared for him to the end. I don't feel worthless.
What you enjoy	My grandkids. Karaoke at my sister's house; I sang his song and laughed and cried at the same time.
Thoughts of suicide	No, never. I want to see my grandson finish high school.
Alcohol, drugs, medications	No alcohol. A cholesterol pill. Nothing for sleep.
Medical history	High cholesterol. My checkup today was fine, and my blood tests last month were normal.
Family history	No one in my family had depression that I know of. My mother grieved my father a long time, in the old way, with the altar and the ceremonies.
Daily life	I watch the grandkids three days a week, go to temple on Sundays, and pay the bills. Hung always did the bills. I learned.
Culture and faith	We keep an altar for our ancestors, and now Hung's photo is there. We had the 49-day and 100-day ceremonies at the temple. That is how we care for our dead, and it helps me.

Only if asked (don’t volunteer)

- The death was four months ago
- Self-worth intact; no suicidal thoughts
- Sadness comes in waves tied to reminders
- Fully caring for her grandchildren and handling the household finances

Patient 28: answer and debrief

DIAGNOSIS

No mental disorder (normal or expected reaction)

Daily tears, weight loss and poor sleep after a death look like depression, and talking to his photo every morning looks like prolonged grief. Clue 3 flips it: four months is far short of the 12 months prolonged grief disorder needs in adults. Her sadness comes in waves tied to reminders, her self-worth and enjoyment are intact, she functions fully, and the altar and memorial ceremonies fit her family's religious and cultural practice. Exam lesson: normal grief is not a disorder; record uncomplicated bereavement with a Z code if useful, and diagnose MDD only if full criteria are met.

What had to be ruled out

- Prolonged grief disorder: only four months since the death; adults need at least 12 months
- Major depressive disorder: sadness comes in waves; self-worth and enjoyment intact; no suicidal thoughts; small weight change
- Adjustment disorder: not used for a normal grief reaction; her response is proportionate and she functions fully
- Medical cause of weight loss: her doctor reviewed it and is not concerned

Checkpoint: What would you ask or check NEXT?

A. Refer her to a treatment program for prolonged grief

B. Ask how the sadness comes and goes, how she sees herself, what she still enjoys, and whether she has thoughts of suicide **(best)**

C. Ask her doctor to start an antidepressant for her sleep and weight loss

At four months, prolonged grief disorder cannot be diagnosed in an adult, so A is ruled out by timing. The real question is normal grief versus a major depressive episode, which turns on the pattern of sadness, self-worth, interest and suicide risk. C treats depression before it has been established.

The case in six clues (as the app reveals it)

1. Vy: She cries every day, she's lost weight, and she isn't sleeping well. I'm afraid she's depressed.
2. My husband, Hung, died of liver cancer. I talk to his photo on our altar every morning, and I still keep his slippers by the door.
3. He died four months ago, at home with hospice. Not long ago our family held his hundred-day ceremony at the temple.
4. The crying comes in waves, when I cook his favorite soup or hear his old songs. Then it passes, and I can laugh with my grandkids about how bad he was at karaoke.
5. I don't feel worthless, and I don't wish I were dead; I want to see my grandson finish high school. Some nights I wake up and reach for him, and I've lost about three pounds, which my doctor says is nothing to worry about.
6. Vy: She still watches the grandkids three days a week, goes to temple, and pays all the bills now. Honestly, Mom is doing more than I am.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Depressive disorder due to another medical condition
2. Prolonged grief disorder, No mental disorder (normal or expected reaction), Major depressive disorder
3. No mental disorder (normal or expected reaction), Major depressive disorder, Adjustment disorder
4. No mental disorder (normal or expected reaction), Major depressive disorder
5. No mental disorder (normal or expected reaction), Adjustment disorder
6. No mental disorder (normal or expected reaction)

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder; DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Uncomplicated Bereavement (Z63.4)

Patient 29: The ramp supervisor

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Grant, 39, an airline ramp supervisor and father of two, at a behavioral health visit in a community health center.

> *"My doctor said my blood pressure's fine but I look stressed. I am stressed. Who isn't?"*

If asked about…

When did this start?	My wife says since our first kid was born, six years ago. I've always been a bit of a worrier, but it got constant then. The layoffs just made it louder.
What you worry about	Everything. Money, the kids getting sick, my mom driving at night, the roof, the weather. It's a new thing every day.
Can you stop it?	No. I tell myself it's pointless, and it keeps going. More days than not.
Your body	Tight shoulders, I grind my teeth, I'm wiped out by afternoon. I lose my place when I'm talking.
Sleep	It takes me an hour or two to fall asleep. My brain runs through every bad scenario.
Sudden attacks	Never had one of those where I thought I was dying. It's more of a steady hum.
Mood and interest	I'm not depressed. I love fishing with the kids. I'm just tense and cranky.
Alcohol, caffeine, drugs, medications	One coffee in the morning, no energy drinks. A beer maybe once a week. No drugs, no prescriptions.
Medical history	Physical last month: thyroid bloodwork and an EKG, both normal. Otherwise healthy.
Thoughts of suicide	No, never. I've got too much to take care of.
Family history	My dad was the same way. He called it being prepared.
Work, family and culture	In my family you tough it out and don't complain. At work I'm the guy who thinks of everything that can go wrong. At home my wife says I'm there but not really there.

Only if asked (don’t volunteer)

- Worry has been constant for about six years, long before the layoffs
- Normal thyroid bloodwork and EKG last month
- Light caffeine and alcohol only
- No sudden panic attacks

Patient 29: answer and debrief

DIAGNOSIS

Generalized anxiety disorder

Clue 2 makes this look like a reaction to the layoffs, which would point to adjustment disorder. Clue 3 flips it: the worry has jumped from topic to topic for six years, well past the 6-month threshold. With caffeine, thyroid and heart ruled out, he has hard-to-control worry plus more than 3 of the linked symptoms (fatigue, poor sleep, irritability, muscle tension, poor concentration, feeling keyed up), with no discrete panic attacks and no loss of interest. Exam lesson: GAD is worry about many real-life matters, more days than not, for 6 months or more; a GAD-7 can track severity.

What had to be ruled out

- Adjustment disorder: the worry began years before the layoffs and meets GAD criteria
- Panic disorder: no sudden attacks
- Major depressive disorder: enjoys fishing with his kids, not hopeless
- Anxiety disorder due to another medical condition: normal thyroid bloodwork and EKG
- Substance cause: one coffee a day, rare alcohol, no other substances
- Obsessive-compulsive disorder: worries about real-life matters, with no rituals

Checkpoint: What would you ask or check NEXT?

A. Reassure him that worrying about a possible layoff is normal

B. Ask about caffeine, energy drinks, alcohol and other substances, and whether a doctor has checked his thyroid and heart **(best)**

C. Teach a worry-postponement exercise and schedule weekly CBT

Before naming an anxiety disorder, rule out substances and medical causes such as hyperthyroidism or a heart rhythm problem. A ignores the six-year pattern in clue 3. C is a sound treatment, but it comes after the assessment.

The case in six clues (as the app reveals it)

1. I'm tired all the time, I can't turn my brain off at night, and I've been short with my kids.
2. The airline announced layoffs about six weeks ago, and I keep running the numbers on what happens if I'm cut.
3. But honestly, my wife says I've been like this most days since our first kid was born, six years ago. Before the layoffs it was the kids getting sick, my mom driving at night, the roof, the weather on every shift.
4. I drink one cup of coffee a day and a beer maybe once a week, nothing else. At my physical last month my thyroid bloodwork and EKG were normal.
5. My shoulders and jaw are always tight, I lose my train of thought in meetings, and I'm keyed up like I'm waiting for bad news. I've never had a sudden attack where I thought I was dying.
6. I still like weekend fishing with the kids, and I don't feel hopeless. I just can't stop the what-ifs, even when I tell myself to.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Generalized anxiety disorder, Insomnia disorder
2. Adjustment disorder, Generalized anxiety disorder, Major depressive disorder
3. Generalized anxiety disorder, Major depressive disorder, Adjustment disorder
4. Generalized anxiety disorder, Major depressive disorder, Panic disorder
5. Generalized anxiety disorder, Major depressive disorder
6. Generalized anxiety disorder

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Generalized Anxiety Disorder; Spitzer, Kroenke, Williams & Lowe (2006), Archives of Internal Medicine 166(10), A brief measure for assessing generalized anxiety disorder: the GAD-7

Patient 30: The video visit

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Farah, 29, a former dental hygienist, at a telehealth visit from her living room. Her husband, Sami, sits beside her.

> *"Sami set this up. I can do video. I can't really do the drive to an office."*

If asked about…

When did this start?	Almost a year ago, on a crowded train after work. Since then my world has gotten smaller and smaller.
What happened that day?	My heart raced, I got dizzy and sweaty, and I was sure I'd pass out. I couldn't get off until the next stop. Maybe ten minutes. I had skipped lunch that day.
Has it happened again?	Not like that. I get a little lightheaded when I think about going out, but nothing out of the blue. At home I'm calm.
Places you avoid	Buses, trains, movie theaters, lines at the bank or pharmacy, big stores. Anywhere I can't leave fast. With Sami I can sometimes do a short trip to my sister's. Alone, not past the mailbox.
What you fear	That I'll get dizzy and faint and nobody will help, or I won't be able to get out. Not what people think of me.
Sleep and mood	I sleep fine. I'm frustrated, not hopeless. I still enjoy cooking and video calls with my friends.
Alcohol, caffeine, drugs, medications	I don't drink. I quit coffee months ago. No drugs, no medications.
Medical history	A heart monitor for two weeks, a blood sugar test, and an ear doctor checked my balance. All normal.
Thoughts of suicide	No. I want my life back.
Work	I left my job at the dental office about eight months ago because I couldn't get there.
Family history	My aunt barely left her house for years. Nobody ever called it anything.
Culture and family	My mother thinks it's nazar, the evil eye, and had prayers said for me. Sami just wants me to see someone. I'm open to both.

Only if asked (don’t volunteer)

- Only one attack, almost a year ago; none since
- Avoids buses, trains, theaters, lines and big stores
- Can manage short trips with Sami
- Normal heart monitor, blood sugar and inner ear tests

Patient 30: answer and debrief

DIAGNOSIS

Agoraphobia

Clue 2 makes panic disorder the lead, but clue 3 flips it: one attack, never repeated and never out of the blue at home, does not meet panic disorder. For almost a year she has feared and avoided buses, trains, theaters and lines because escape or help might not be available, and she copes better with a companion. Normal heart, blood sugar and inner ear tests rule out a medical cause, and fear of judgment is not the issue. Exam lesson: agoraphobia is diagnosed separately from panic disorder and needs fear of 2 or more of 5 situation types for 6 months or more.

What had to be ruled out

- Panic disorder: a single attack almost a year ago; no recurrent unexpected attacks
- Social anxiety disorder: comfortable being watched; the fear is of being trapped or without help
- Specific phobia: the fear spans several types of situations, not one
- Anxiety disorder due to another medical condition: normal heart monitor, blood sugar and inner ear tests
- Substance cause: no alcohol; stopped coffee months ago

Checkpoint: What would you ask or check NEXT?

A. Explain that she has panic disorder and begin breathing retraining

B. Ask whether a doctor has checked the racing heart and dizziness, and about caffeine or other substances **(best)**

C. Encourage her to ride the train alone this week to face the fear

A racing heart and dizziness need a medical and substance rule-out first, such as a heart rhythm problem, low blood sugar, an inner ear problem or caffeine. A labels panic disorder from a single attack, and that diagnosis needs recurrent unexpected attacks. C jumps to exposure before assessment; exposure is planned, graded and agreed with the client.

The case in six clues (as the app reveals it)

1. I had to leave my job, and I haven't been inside a grocery store in almost a year. Sami does all the shopping now.
2. It started on a packed train. My heart raced, I got dizzy, and I was sure I'd faint with no way off and nobody to help me.
3. That's the only time it ever hit me like that, and it has never come out of nowhere at home. What scares me is that it could happen again somewhere I can't get out of.
4. Sami: She won't take the bus or the train, she left a movie halfway through, and she can't stand in a line at the bank. If I'm with her, she can sometimes manage a short trip.
5. It isn't about people watching me; I gave a toast at my sister's dinner last month with everyone looking at me, and I was fine. It's being stuck somewhere if I get dizzy and pass out.
6. After that day a cardiologist had me wear a heart monitor, and my doctor checked my blood sugar and my inner ear. Everything was normal, I don't drink, and I quit coffee months ago.

What a skilled clinician weighs after each clue

1. Agoraphobia, Social anxiety disorder, Major depressive disorder
2. Panic disorder, Agoraphobia, Specific phobia
3. Agoraphobia, Specific phobia, Social anxiety disorder
4. Agoraphobia, Social anxiety disorder
5. Agoraphobia, Anxiety disorder due to another medical condition
6. Agoraphobia

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Agoraphobia; DSM-5-TR (APA, 2022), Anxiety Disorders: Panic Disorder

Patient 31: The cracked hands

Setting: Svetlana, 34, a hospital lab technician, at a county behavioral health clinic. The employee health nurse referred her for damaged skin on her hands. Her husband, Pavel, came with her.

> *"The nurse at work sent me because of my hands. They're cracked and bleeding. I'm only here so I can keep my job."*

If asked about…

When did this start?	About three years ago, after my daughter was in the NICU for two weeks with an infection. She's fine now. I haven't been fine since.
What goes through your mind	Pictures of germs on her hands, in her mouth. They pop in a hundred times a day. I know other moms don't think like this.
What you do about it	I wash in sets of seven. Shoes off in the garage, groceries wiped, Pavel's clothes washed twice. If I'm interrupted, I have to start over.
How much time	Four or five hours a day. Getting out the door takes two.
How sure are you?	I'm pretty sure if I stop, she'll get sick and die. Pavel says that's crazy. Some days I think maybe I go overboard. Most days I think I'm right.
Unusual experiences	No voices, nobody following me, no messages from the TV. It's just germs.
Perfectionism and order	I used to be the messy one! I don't care if the towels are folded right. I care if they're clean.
Sleep and mood	I get to bed late because the bedtime routine takes so long. I'm worn down, and sometimes I cry because I can't stop.
Alcohol, drugs, medications	Maybe a glass of wine at New Year's. No drugs. A cream for my hands.
Medical history	Normal checkup this year. The dermatologist says the skin damage is from the washing.
Thoughts of suicide or harming anyone	No. I would never hurt myself or my daughter. That's the whole point; I'm trying to protect her.
Family history and culture	My mother kept a very clean house, but nothing like this. I came from Kyiv at nineteen. In my family you don't go to therapists; my grandmother always said you handle things yourself.

Only if asked (don’t volunteer)

- Rituals take four to five hours a day
- Began about three years ago after her daughter's NICU stay; she was easygoing before
- Fairly sure her daughter will die if she skips a step, though sometimes doubts it
- No hallucinations or other unusual beliefs

Patient 31: answer and debrief

DIAGNOSIS

Obsessive-compulsive disorder

Clue 2 sounds like a rigid, perfectionist personality, but clue 3 flips it: she hates the rules, and intrusive images drive washing rituals done to ease distress. Clue 4 raises psychosis, but the belief stays on the OCD theme, she has no hallucinations or other delusions, and she can sometimes doubt it, so the specifier is poor insight. A clear start three years ago, after being easygoing, and four to five hours a day rule out a lifelong personality pattern. Exam lesson: even absent insight with delusional conviction is an OCD specifier, not a psychotic disorder; first-line care is exposure and response prevention (ERP) and an SSRI.

What had to be ruled out

- Obsessive-compulsive personality disorder: the rituals feel unwanted, and they began three years ago rather than as a lifelong pattern
- Delusional disorder or other psychosis: the belief is limited to the contamination theme, she can sometimes doubt it, and there are no hallucinations
- Generalized anxiety disorder: intrusive images with rituals, not everyday real-life worry
- Substance or medical cause: no alcohol or drug use; normal checkup apart from skin damage

Checkpoint: What would you ask or check NEXT?

A. Refer her for an antipsychotic evaluation because the belief sounds fixed

B. Ask how firmly she holds the belief, whether she can ever doubt it, and whether she has hallucinations or other unusual beliefs **(best)**

C. Correct the belief directly with facts about germs and infection rates

Strong conviction can be an OCD insight specifier rather than psychosis, so assess how fixed the belief is and whether there are other psychotic symptoms before deciding. A assumes a psychotic disorder without that assessment. C invites argument and reassurance-seeking; it does not help the diagnosis, and reassurance tends to feed the cycle.

The case in six clues (as the app reveals it)

1. I'm late to work almost every day, because getting out the door takes me two hours. I'm exhausted all the time.
2. Pavel: She has rules for everything now. Shoes off in the garage, groceries wiped down, my clothes washed twice, and if I skip a step she gets furious. She's become a total perfectionist.
3. I hate the rules. A picture of germs crawling on my daughter's hands jumps into my head over and over, and I can't calm down until I've washed in sets of seven.
4. And honestly, I think I'm probably right. If I skip a step, I'm fairly sure she'll get sick and die. Everyone else is just careless.
5. I don't hear voices, and nobody is out to get us; it's just germs, and some days I admit I may go overboard. My checkup was normal except for my cracked hands, and I don't drink or use anything.
6. Pavel: It eats four or five hours a day. It started about three years ago, after our daughter spent two weeks in the NICU; before that, she was the messy one in our house.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Obsessive-compulsive disorder, ADHD
2. Obsessive-compulsive personality disorder, Obsessive-compulsive disorder, Generalized anxiety disorder
3. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder, Generalized anxiety disorder
4. Obsessive-compulsive disorder, Delusional disorder
5. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder
6. Obsessive-compulsive disorder

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder; Koran & Simpson (2013), American Psychiatric Association, Guideline Watch: Practice Guideline for the Treatment of Patients With Obsessive-Compulsive Disorder

Patient 32: The hallway

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Harold, 81, a retired newspaper typesetter and widower, on a hospital rehabilitation unit after a fall at home. His daughter, Carol, joins the discharge planning meeting, and his occupational therapist adds notes.

> *"I'm going home. My daughter thinks I need a nursing home. I need my house and my papers."*

If asked about…

When did the saving start?	About fifty years ago, when my wife and I married and got a house with room. She kept it to the garage. After she died six years ago, it spread.
What you keep and why	Newspapers, magazines, clippings. Every one might have something I'll need. Throwing one out is like burning a library.
Getting rid of things	Carol hauled the hallway to the curb three years ago. I couldn't sleep, and I brought it all back in that night.
Getting more	I pick up the free papers everywhere I go. The library sale every week.
The house	The hallway is tight, I'll grant you. The kitchen table is covered, the spare room too. I eat in my chair.
The stove and the bills	A stack was in front of the knob, so I didn't see it was on. The bills got buried. I found them later and paid them.
Memory	My memory's fine. I did that test here, and the therapist said I did well. I do my crossword in pen.
Sleep and mood	I sleep fine in my chair. I miss my wife, but I'm not depressed. I like my radio and my papers.
Alcohol, drugs, medications	A beer with the ballgame. A blood pressure pill and a baby aspirin. I handle them myself.
Medical history	The fall cracked my pelvis. Blood pressure. Otherwise healthy.
Thoughts of suicide	No. I want to go home.
Family and culture	My parents lived through the Depression. You didn't throw anything away; my father saved string. Carol is a good girl, but she doesn't understand.

Only if asked (don’t volunteer)

- Saving began about fifty years ago and spread after his wife's death
- Normal memory, thinking and mood screens; manages his own pills
- Brought the cleared papers back in from the curb
- Picks up free papers daily and goes to the library sale weekly

Patient 32: answer and debrief

DIAGNOSIS

Hoarding disorder

Clue 3 points toward dementia, which in older adults can cause new clutter. Clue 4 flips it back: his cognitive, mood and lab screens are normal, he manages his own pills, and the lapses come from the clutter itself. Saving for about fifty years, distress at discarding, buying and collecting more, and rooms he cannot use point to hoarding disorder with excessive acquisition. Exam lesson: in late-life hoarding, rule out a neurocognitive or medical cause, then address fire, fall and stove risks with the client through harm reduction rather than forced cleanouts.

What had to be ruled out

- Major neurocognitive disorder: normal cognitive screen, manages his own medications, and the saving began decades before any lapses
- Major depressive disorder: normal mood screen; enjoys his radio and papers
- Obsessive-compulsive disorder: saving is not driven by contamination or harm obsessions
- Medical cause: normal thyroid and B12

Checkpoint: What would you ask or check NEXT?

A. Arrange a cognitive and mood screen and a medical review before deciding what is driving the clutter and the lapses **(best)**

B. Recommend a nursing home, since he cannot live safely alone

C. Arrange a full cleanout of the house before he is discharged

New lapses in an 81-year-old could mean a neurocognitive disorder, which would change both the diagnosis and the plan, so screen cognition, mood and medical causes first. B jumps to the most restrictive option before assessment and without his input. C, a forced cleanout, is distressing, rarely lasts and can destroy trust; safety work is done with him.

The case in six clues (as the app reveals it)

1. Carol: Dad fell at home and lay on the floor for hours before a neighbor heard him. The paramedics could barely get the stretcher through the house.
2. Carol: The hallway is lined with stacks of newspapers and magazines up to his shoulders. Nobody has eaten at the kitchen table or slept in the spare room in years.
3. Carol: This past year he left the stove on twice and missed two electric bills. I think his mind is going.
4. Occupational therapist: His memory, thinking and mood screens are normal, and so are his thyroid and B12; he manages his own pills here without help. He says the bills got buried in the piles and a stack hid the stove knob.
5. I've kept them since my wife and I married, about fifty years ago. Every paper might have something I'll need, and throwing one out feels like burning a piece of history.
6. I know it's too much. When Carol cleared the hall three years ago, I couldn't sleep, and I hauled it all back in from the curb that night; I still pick up a free paper everywhere I go.

What a skilled clinician weighs after each clue

1. Major neurocognitive disorder, Hoarding disorder, Major depressive disorder
2. Hoarding disorder, Major neurocognitive disorder, Obsessive-compulsive disorder
3. Major neurocognitive disorder, Mild neurocognitive disorder, Hoarding disorder
4. Hoarding disorder, Obsessive-compulsive disorder
5. Hoarding disorder, Obsessive-compulsive personality disorder
6. Hoarding disorder

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Hoarding Disorder; Tompkins & Hartl (2009), Digging Out (New Harbinger), Harm reduction approach to hoarding

Patient 33: The grip test

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Kwabena (Kobby), 30, an insurance claims adjuster, at a behavioral health visit in his primary care clinic. His fiancee, Ama, came with him.

> *"My doctor says the twitches are nothing. Then why do they keep happening? I need someone to be sure."*

If asked about…

When did this start?	About ten months ago. My coworker's dad died of ALS. A week later I noticed my calf twitching.
The twitches	Little flickers under the skin. They don't hurt. I can run fine. I mostly notice them when I'm checking for them.
What you fear	ALS. I've read everything. I know the tests were normal, but maybe they missed it because it's early.
Checking and searching	I test my grip every night and watch videos of patients to compare. Maybe two hours a night.
Doctor visits	Two neurologists, a nerve test, bloodwork. All normal. I feel better for about a week, then the doubt comes back and I book another appointment.
Other worries	Not really. Money's fine, work's fine. It's just this.
Sleep and mood	I'm up late searching. I'm not depressed. I play pickup soccer on Saturdays and I still enjoy it.
Alcohol, drugs, medications	A drink at weddings, that's it. No drugs. A multivitamin and magnesium because I read it helps twitches.
Medical history	Healthy. Normal physical, normal labs.
Thoughts of suicide	No. I want to live; that's why I'm so scared.
Family history	My mom worries a lot about health. She calls me after every doctor visit to hear the results.
Culture and relationships	Our wedding is in five months, and Ama is tired of hearing about this. In my family you see the doctor and you trust the doctor, so I feel ashamed that I can't.

Only if asked (don’t volunteer)

- The twitches are mild and not painful
- The fear began about ten months ago after a coworker's father died of ALS
- Normal nerve test; relief lasts about a week
- Tests his grip and watches patient videos every night

Patient 33: answer and debrief

DIAGNOSIS

Illness anxiety disorder

Twitching and repeated specialist visits suggest somatic symptom disorder, but clue 3 flips it: the symptom is mild and barely noticed, and the distress is about having a deadly disease. Normal tests bring only brief relief, he checks every night, and the fear has lasted ten months, past the 6-month threshold, with no broad worry, panic or depression. This is illness anxiety disorder, care-seeking type. Exam lesson: in somatic symptom disorder the symptom itself is distressing; in illness anxiety disorder symptoms are absent or mild and the fear of illness drives the problem. Regular scheduled visits with one doctor and CBT help.

What had to be ruled out

- Somatic symptom disorder: the twitches are mild and not distressing in themselves
- Generalized anxiety disorder: worry is limited to one illness
- Obsessive-compulsive disorder: no other obsessions or rituals; the checking serves only his fear of illness
- Panic disorder: no attacks
- Major depressive disorder: still enjoys soccer; not depressed
- Neurological disease: two normal neurology exams and a normal nerve test

Checkpoint: What would you ask or check NEXT?

A. Refer him for a third neurology opinion to settle his fear

B. Ask how much the twitches themselves bother him, compared with his fear of what they mean, and how much time he spends checking **(best)**

C. Tell him the twitches are just anxiety and he should stop seeing doctors

What separates somatic symptom disorder from illness anxiety disorder is whether distress centers on a bothersome symptom or on fear of disease when symptoms are mild, so ask. A feeds the cycle of seeking reassurance when results so far are normal; care is better coordinated through one primary care doctor. C is dismissive, harms the relationship, and he still needs a medical home.

The case in six clues (as the app reveals it)

1. I can't focus at work, I'm up half the night on my phone, and I'm in this clinic more than anyone my age.
2. The muscles in my calves twitch. I've seen two neurologists and had a nerve test, and I've read that ALS can start this way.
3. Honestly, the twitches are tiny. They don't hurt, they don't slow me down, and I barely notice them unless I'm checking; it's what they might mean that I can't let go of.
4. Ama: Every night he tests his grip and watches videos of patients. The nerve test was normal, he felt better for about a week, and then he booked another specialist.
5. I don't really worry about money or work or anything else. It's only this, and it started about ten months ago, after my coworker's dad died of ALS.
6. I've never had a sudden panic attack, and I don't drink or use anything. I'm not depressed; I still play pickup soccer on Saturdays.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Somatic symptom disorder, Major depressive disorder
2. Somatic symptom disorder, Illness anxiety disorder, Generalized anxiety disorder
3. Illness anxiety disorder, Somatic symptom disorder, Generalized anxiety disorder
4. Illness anxiety disorder, Obsessive-compulsive disorder, Somatic symptom disorder
5. Illness anxiety disorder, Obsessive-compulsive disorder
6. Illness anxiety disorder

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Illness Anxiety Disorder; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Somatic Symptom Disorder

Patient 34: The fourth hospital

Setting: Wen, 39, a bank teller, on a hospital medicine unit. The hospitalist asks for a social work consult, and her mother visits each afternoon.

> *"Are you from psychiatry? The doctors still haven't found what's wrong with me. I hope they keep looking."*

If asked about…

When did this start?	About a year ago. Four hospitals this year. Each time I get shaky and sweaty and wake up in the ER.
What the doctors found	My sugar is always very low. They keep saying it's strange because I'm not diabetic. I want them to find the cause, whatever it takes.
Insulin at home	My mom has diabetes. I help her with her pens sometimes. Why do you ask?
Work and money	I'm a teller. I've used up my sick days. I'm not suing anybody and I don't want disability. I want answers.
Life outside the hospital	Quiet. I've lived with my mom since my divorce two years ago. Work, home, TV. Everyone here is so kind to me.
Sleep and mood	I sleep okay at home. I get lonely sometimes. In the hospital, at least people check on me.
Alcohol, drugs, medications	I don't drink. No drugs. No prescriptions of my own.
Medical history	As a teenager I was in the hospital a lot for stomach pain they never explained. Otherwise healthy.
Thoughts of suicide	No. I don't want to die. I've never tried to hurt myself. I want to get better.
Family history	My mom has diabetes and heart trouble. My dad left when I was little. Nobody in my family has seen a therapist.
Culture and family	My mother says we don't complain to outsiders, so I don't. She has come every afternoon this week. It's the most time we've spent together in years.

Only if asked (don’t volunteer)

- Has access to her mother's insulin pens
- Records from three other hospitals show the same pattern
- No lawsuit, disability claim or other external gain
- Feels cared for and less lonely in the hospital

Patient 34: answer and debrief

DIAGNOSIS

Factitious disorder imposed on self

Clue 2 points to a medical cause, such as an insulin-making tumor. Clue 3 flips it: high insulin with low C-peptide means the insulin came from outside the body, and her mother's missing pens fit. She seeks no money, claim or drugs and denies suicidal intent, which rules out malingering, and deliberate deception rules out somatic symptom disorder. Exam lesson: factitious disorder means deliberately falsifying or inducing illness with no obvious external reward. Keep her medically safe, coordinate one care team, and share the findings in a supportive, non-punitive way.

What had to be ruled out

- Malingering: no lawsuit, disability claim, drugs or other external gain
- Somatic symptom disorder: the illness is deliberately induced
- Medical cause such as an insulin-making tumor: low C-peptide shows the insulin came from outside the body
- Suicidal behavior: denies intent to die, no past attempts; keep reassessing

Checkpoint: What would you ask or check NEXT?

A. Tell her the lab results prove she is faking and that she will be discharged

B. Keep her medically safe, ask directly about suicidal intent, and gather more history, such as other hospitals' records and any outside incentives **(best)**

C. Notify her employer's disability insurer that her absences may be fraudulent

Self-induced low blood sugar can kill, so medical safety and a direct question about suicide come first. Records and a search for any external motive then separate factitious disorder from malingering. A is a premature, hostile confrontation that often ends with the client leaving and seeking care elsewhere. C breaches confidentiality with no legal basis.

The case in six clues (as the app reveals it)

1. I keep getting shaky and sweaty, and then I pass out. This is my fourth hospital this year.
2. Hospitalist's note: Her blood sugar has been dangerously low on arrival each time. She does not have diabetes.
3. Hospitalist's note: During the last low, her insulin level was very high but her C-peptide was low, which means the insulin came from outside her body.
4. Mother: I keep losing my insulin pens. I thought I was getting forgetful.
5. I'm not trying to kill myself, if that's what you're asking. I'm not suing anyone or asking for disability, and I've used up my own sick days to be here.
6. Hospitalist's note: Records from three other hospitals show the same pattern. She asks for more tests, agreed at once to a pancreas biopsy, and seems most at ease when the team is rounding on her.

What a skilled clinician weighs after each clue

1. Panic disorder, Somatic symptom disorder, Functional neurological symptom disorder
2. Somatic symptom disorder, Factitious disorder imposed on self, Malingering
3. Factitious disorder imposed on self, Malingering
4. Factitious disorder imposed on self, Malingering
5. Factitious disorder imposed on self, Malingering
6. Factitious disorder imposed on self

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Factitious Disorder (Imposed on Self); DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Malingering

Patient 35: The cross-country runner

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tobias, 19, a sophomore cross-country runner, sees the social worker in the university sports medicine clinic. The team physician has sent notes.

> *"Doc says I have to see you to get cleared to run. I'm fine. I just need to fix my times."*

If asked about…

When did this start?	About a year ago I started cutting out junk to get faster. I've lost about 30 pounds since then. The nights when I lose control started about four months ago.
Eating pattern	Most days I barely eat. About twice a week I lose control and eat a ton, then I throw up.
Body image	I still see a soft stomach. People say I'm too thin. They don't see what I see.
Weight	Doc says 112. I'd like to be lighter for racing. Gaining scares me more than anything.
Physical symptoms	I'm always cold. I got dizzy standing up at practice last week. My teeth are sensitive.
Sleep and mood	I wake up at four, hungry, and go run. I'm not depressed. I'm irritable when I'm hungry. Food is the one thing I control.
Alcohol, drugs, medications	No drinking during season. No drugs, no laxatives, no diet pills.
Medical findings	Doc said my potassium was low and my heart rate was 38. He looked worried.
Thoughts of suicide or self-harm	No. I've never thought about killing myself or hurting myself. I just want to run well.
Family history	My mom is always on some diet. My uncle drinks too much.
Team and culture	In distance running, lighter is faster; everybody says it. A guy with an eating problem? My teammates would never get it.

Only if asked (don’t volunteer)

- Vomits after binges about twice a week for four months
- Barely eats on other days and runs extra miles
- Low potassium and a resting heart rate of 38
- Lost about 30 pounds in a year

Patient 35: answer and debrief

DIAGNOSIS

Anorexia nervosa

Clues 2 and 3 describe binges and vomiting at least weekly for months, which looks like bulimia nervosa. Clue 4 flips it: a BMI of about 16 is significantly low, and when binge-purge behavior happens during anorexia, the diagnosis is anorexia nervosa, binge-eating/purging type (moderate by BMI). Fear of weight gain and a distorted body image complete the picture. Low potassium and a heart rate of 38 are medical red flags that need urgent medical care, often in the hospital; he should not be cleared to train until medically stable, and refeeding must be medically monitored. Exam lesson: weight separates the two; anorexia has one of the highest death rates in psychiatry, and it is often missed in men.

What had to be ruled out

- Bulimia nervosa: his weight is significantly low, and binge-purging during anorexia is diagnosed as anorexia
- Binge-eating disorder: he purges after binges
- Avoidant/restrictive food intake disorder: fear of weight gain and body-image disturbance are present
- Major depressive disorder: not sad most days
- Substance cause: no drugs, laxatives or diet pills

Checkpoint: What would you ask or check NEXT?

A. Refer him to a CBT group for bulimia at the counseling center

B. Arrange a prompt medical evaluation of his weight, vital signs and blood electrolytes **(best)**

C. Ask his coach to watch his meals and report back to you

Vomiting plus restriction in an athlete who is cold and declining can cause dangerous electrolyte and heart problems, and the diagnosis turns on his weight, so medical evaluation comes first. A assumes bulimia before his weight is known. C breaks confidentiality and puts the coach in a monitoring role without his consent.

The case in six clues (as the app reveals it)

1. My race times keep getting worse, I'm cold all the time, and Coach says I look wiped out.
2. Some nights I eat a whole box of cereal and a sleeve of cookies and can't stop. Then I make myself throw up.
3. It's been about twice a week for the past four months. On the other days I barely eat, and I run extra miles.
4. Team physician's note: He is 5 feet 10 inches and 112 pounds, a BMI of about 16, down 30 pounds in a year. His potassium is low and his resting heart rate is 38.
5. When I look in the mirror I still see a soft stomach. If I gained even five pounds, I'd quit the team.
6. I'm not sad most days, and I've never thought about killing myself. I don't use drugs, laxatives or diet pills; food is the one thing I feel I control.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Anorexia nervosa, Avoidant/restrictive food intake disorder
2. Bulimia nervosa, Anorexia nervosa
3. Bulimia nervosa, Anorexia nervosa
4. Anorexia nervosa, Avoidant/restrictive food intake disorder
5. Anorexia nervosa
6. Anorexia nervosa

Sources: DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Anorexia Nervosa; American Psychiatric Association (2023), Practice Guideline for the Treatment of Patients With Eating Disorders, 4th ed.

Patient 36: The sixth grader's lunch

Setting: Adaeze, 12, a sixth grader, meets the school social worker after her teacher noticed she skips lunch. Her mother, Ngozi, joins the meeting.

> *"My mom made me come. I'm not starving myself. I just can't eat hard food."*

If asked about…

When did this start?	Five months ago, at my cousin's graduation dinner. A piece of steak got stuck, and my dad had to do the Heimlich. Since then I only eat soft stuff.
What you eat	Yogurt, soup, smoothies, mashed potatoes, and the shakes the doctor gave me. Nothing I have to chew a lot.
What you fear	That it'll get stuck again and I won't be able to breathe. My throat gets tight just looking at chicken.
Weight and body	I want to gain weight. I'm always cold, and I miss pizza. I don't think I'm fat.
Friends and school	I sit with my friends at lunch and say I'm not hungry. I skip sleepovers because of the pizza. At my best friend's house I eat soup.
Sleep and mood	I sleep fine. I get sad that I'm missing stuff, but I still like drawing and basketball.
Medical history	Mother: The pediatrician says she's gone from the 50th to the 15th percentile for weight. The throat doctor's swallow X-ray and scope were normal. Otherwise she's healthy.
Medications and substances	Just the nutrition shakes. No medicines. I've never tried anything like that.
Thoughts of hurting yourself	No. I'm just scared of choking.
Family history	Mother: My sister was a picky eater as a child. No eating disorders or anxiety problems in the family that I know of.
Culture and family	Mother: Sunday dinner is a big deal for us, jollof rice, everybody at the table. Her grandmother thinks she's being stubborn. I just want my daughter back at the table.

Only if asked (don’t volunteer)

- Choked at a family dinner five months ago
- Wants to gain weight; no body-image concern
- Dropped from the 50th to the 15th weight percentile; on nutrition shakes
- Normal swallow X-ray and scope

Patient 36: answer and debrief

DIAGNOSIS

Avoidant/restrictive food intake disorder

Weight loss, skipped lunches and baggy clothes in a 12-year-old girl point first to anorexia. Clue 3 flips it: the restriction began right after a choking scare, and clues 4 and 5 show she wants to gain weight and has no body-image concern. Falling off her growth curve and needing nutrition shakes, with normal swallow studies, make the eating problem itself the focus, so this is ARFID rather than a choking phobia alone; DSM-5-TR prefers ARFID once the eating problem becomes the main focus of care. Exam lesson: ARFID has no fear of fatness or body-image disturbance; it can follow choking or vomiting, and care combines medical monitoring with graded exposure to feared foods, as in CBT for ARFID.

What had to be ruled out

- Anorexia nervosa: no fear of weight gain and no body-image disturbance; she wants to gain
- Social anxiety disorder: she eats soup in front of friends; the avoidance is about food, not people
- Specific phobia: weight loss and dependence on supplements make the eating disturbance the focus
- Medical swallowing problem: normal swallow X-ray and scope
- Major depressive disorder: still enjoys drawing and basketball; sleeps well

Checkpoint: What would you ask or check NEXT?

A. Ask what she worries would happen if she ate a regular lunch, and how she feels about her body and weight **(best)**

B. Refer her to an eating disorder program for anorexia

C. Set up a plan for her teacher to watch her eat lunch every day

Weight loss and baggy clothes raise anorexia, but the reason she avoids food decides the diagnosis, so ask about her fears and body image first; a pediatric medical check is also needed. B labels her before that is known. C imposes control without understanding the problem and singles her out in front of peers.

The case in six clues (as the app reveals it)

1. Mother: She's lost weight, she won't eat lunch at school, and she's stopped going to sleepovers.
2. I wear my brother's hoodies now. At lunch I just sit with my friends and say I'm not hungry.
3. Five months ago I choked on a piece of steak at my cousin's graduation dinner, and my dad had to do the Heimlich.
4. I'm not trying to get skinny; I want to gain weight and eat pizza with my friends. I just know food will get stuck, so I only have yogurt, soup and smoothies.
5. The hoodies are because I'm cold all the time, not because of how I look. I skip sleepovers because of the pizza, not the people; at my best friend's house I eat soup in front of everybody.
6. Mother: The pediatrician says she has dropped from the 50th to the 15th percentile for weight and put her on nutrition shakes. The throat doctor did a swallow X-ray and a scope, and both were normal.

What a skilled clinician weighs after each clue

1. Anorexia nervosa, Major depressive disorder, Social anxiety disorder
2. Anorexia nervosa, Social anxiety disorder, Avoidant/restrictive food intake disorder
3. Avoidant/restrictive food intake disorder, Specific phobia, Anorexia nervosa
4. Avoidant/restrictive food intake disorder, Specific phobia
5. Avoidant/restrictive food intake disorder, Specific phobia
6. Avoidant/restrictive food intake disorder

Sources: DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Avoidant/Restrictive Food Intake Disorder; Thomas & Eddy (2019), Cambridge University Press, Cognitive-Behavioral Therapy for Avoidant/Restrictive Food Intake Disorder